4/13/2026

TO: Billing Department / Claims Administrator
Mayo

RE: FORMAL BILLING DISPUTE & REQUEST FOR RECONSIDERATION

Patient Name: John Doe
Account/Invoice: 1351518 | Insurance ID: 1684811
Statement Date: 2026-04-13

I am writing to formally dispute the charges on the above-referenced invoice. I have identified several billing irregularities, referenced against specific CMS Medicare Claims Processing Manual chapters, that require immediate correction. Pursuant to 45 CFR §164.524, I request an itemized statement and all supporting documentation within 30 days.

SPECIFIC REASONS FOR DISPUTE (CMS-REFERENCED):

Ø=Ý B U N D L I N G (N C C I)

CMS Reference: Per CMS National Correct Coding Initiative (NCCI) — Medicare Claims Processing Manual, Chapter 23, §20. This code pair constitutes a Column 1/Column 2 edit; the component code is not separately billable.

• NCCI Bundle Violation: Hard Bundle: 80053 is a component of 80048 and NO modifier can bypass this edit per CMS. Date: 04/10/26. (CMS NCCI Edits, Chapter 1)

Ø=ÜÊ F R E Q U E N C Y (M U E)

CMS Reference: Per CMS Medically Unlikely Edit (MUE) limits — Medicare Claims Processing Manual, Chapter 1, §70. The daily unit limit for this code has been exceeded. Excess units are non-payable.

• Line Item Frequency Violation (MUE MAI 1): CPT 93000 billed at 4 units. CMS MUE limit for this line item is 1. (MAI 1: Line Item Adjudication)

&–þ F I N A N C I A L I N T E G R I T Y

CMS Reference: Per CMS credit balance reporting requirements — Medicare Claims Processing Manual, Chapter 4, §50. Credit balances must be reported and refunded within 60 days.

• Invoice Reconciliation Gap: Invoice sum: Billed ($26980.00) does not balance with Paid+Owed+Adj ($650.00). A $26330.00 discrepancy remains.

• Likely Silent Denial / Process Oversight: Line item "Manual Entry" billed at $650.00 passed directly to patient with $0 insurance payment and $0 adjustment. Verify EOB for this charge.

Ø>Þz CLINICAL CONTEXT

CMS Reference: Per CMS Medical Necessity documentation requirements — Medicare Claims Processing Manual, Chapter 13, §10. Procedures must be clinically appropriate for the reported diagnosis.

• Isolated Procedure — Missing Clinical Support: ECG (93000) billed without any expected support codes on 04/10/26. US Medicare guidelines suggest this procedure should exist within a high-level diagnostic context.

Ø=ÜÍ PLACE OF SERVICE

CMS Reference: Per CMS Place of Service (POS) coding — Medicare Claims Processing Manual, Chapter 26, §10.5. The place of service code must match the actual setting in which the service was rendered.

• Possible Institutional Code Mismatch: Hospital Inpatient CPT 99231 detected while in "Office/Clinic" mode. This is a Place-of-Service (POS) mismatch per CMS Claims Processing Manual Chapter 26 §10.5. Verify if service was actually provided in a hospital (POS 21).

Ø=Ý MODIFIER AUDIT

CMS Reference: Per CMS modifier documentation requirements — Medicare Claims Processing Manual, Chapter 12, §30.6. This surgical procedure requires specific modifiers to be separately payable.

• Multiple Procedure Reduction Missing (-51): Multiple surgical procedures (2) billed on 04/10/26 without a -51 modifier. Primary payers typically discount the second procedure by 50%. Potential overcharge detected.

• Global Package Violation: CPT 99215 (E&M) billed on the same date as a surgery code without a -24, -25, or -57 modifier. May not be separately payable — see CMS Claims Processing Manual Chapter 12.

Ø=ÜË COMPLETENESS

• High-Level E&M — Upcoding Risk: CPT 99215 (Established patient high
documented physician time. Frequently upcoded for short visits. Request visit notes per CMS Claims Processing Manual, Chapter 12 §30.6.

CLINICAL

• High-Level Visit vs. Routine Diagnosis: CPT 99215 (High Complexity) is billed with a routine/wellness diagnosis (Z00.00). Medicare typically expects a lower-level visit (e.g. 99212-3) for routine exams unless documentation supports significant additional complexity. Potential upcoding risk.

Ø=Ü° BENCHMARK PRICING

CMS Reference: Per CMS Medicare Physician Fee Schedule (MPFS) — Medicare Claims Processing Manual, Chapter 12. This charge significantly exceeds the allowable fee schedule amount. Reference: cms.gov/Medicare/Medicare-Fee-for-Service-Payment/PhysicianFeeSched.

• High Pricing Variance: CPT 99215: Billed $650.00 vs 2.5× Medicare benchmark $464.68. Potential overcharge of $185.32. (CMS Medicare Physician Fee Schedule — cms.gov/Medicare/Medicare-Fee-for-Service-Payment/PhysicianFeeSched)

ESTIMATED OVERCHARGE (POTENTIAL RECOVERY): $23453.98

This figure is based on deduplicated, per-line-item savings calculations capped to each line's billed total. It represents a conservative estimate using CMS Medicare benchmark pricing where available.

I request a written response within 30 days identifying which errors will be corrected and the revised balance owed. Failure

Sincerely,

John Doe